جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : جيهان فتحى على محمد
المحافظة : القليوبيه **الحي / المدينة :** شبرا الخيمه **العنوان :** مدخل عزبه الحافي
رقم البطاقة : 27209111400669 **رقم التليفون :** 01221367744 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 11-09-1972 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: جيهان فتحى على محمد
National ID: 27209111400669

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٠١٢٢١٣٦٧٧٤٤
٠١٢٢٥٧٩٨٩٠٣



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International Egypt Pathology Lab
جوستاف روسي

Patient Name	جيهان فتحى على	**Case No.**	**341434**
Sex/Age	**Female 53 Y**	**Patient No.**	**24672**
Registered Date	**11/7/2026 03:11:13 PM**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	**11/8/2026 01:07:51 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**16/8/2026 09:58:54 AM**	**Pathology No.**	**2182/26**

Clinical Data: Bilateral breast masses. BIRADs 4.

Nature of Specimen:
1- Right breast UIQ mass, Tru-cut biopsy, collected on 11/07/2026 at 2:00 pm, Cold ischemia time: 20 seconds.
2- Left breast UOQ mass, Tru-cut biopsy, collected on 11/07/2026 at 2:05 pm, Cold ischemia time: 20 seconds.

Gross Examination: Two containers were received, designated as follow:
1- Right breast mass: Fragmented grey, white tissue cores, ranging in size between 0.2 cm & 0.5 cm long, totally submitted.
2- Left breast mass: Three grey, white tissue cores, ranging in size between 0.2& 0.4 cm long, totally submitted.
Technique:
- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Microscopic Examination:
1- Right breast mass:
- Histologic type: Invasive lobular carcinoma "Minimally represented"
- Histologic grade: 2 (Differentiation 3, pleomorphism 2, mitosis 1).
- LCIS: Present.
- DCIS: Absent.
- Microcalcification: Absent.
- Angioinvasion: Absent.
- Perineural invasion: Absent

*** IPT (All controls showed appropriate reactivity):**
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 10% of tumor cells (score 5/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- SISH: Amplified with clusters.
- Ki67%: 2%.

2- Left breast mass:
- Histologic type: Invasive lobular carcinoma.
- Histologic grade: 2 (Differentiation 3, pleomorphism 2, mitosis 1).
- LCIS: Present.
- DCIS: Absent.

Immunohistochemistry
[[DOCTYPE:IHC]]

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- Microcalcification: Absent.
- Angioinvasion: Absent.
- Perineural invasion: Absent

*** IPT (All controls showed appropriate reactivity):**

- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 20% of tumor cells (score 5/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: No residual cells to evaluate.
- Ki67%: 5%.

Diagnosis:

Right breast mass (1) & Left breast mass (2), Tru-cut biopsies:

1- Right breast mass:

- Invasive lobular carcinoma, "Minimally represented" Grade 2, with lobular carcinoma in situ.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 5/8).
- Her2: Positive (score 3+).
- SISH: Amplified with clusters.
- Ki67%: 2%.

2- Left breast mass:

- Invasive lobular carcinoma, Grade 2, with lobular carcinoma in situ.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 5/8).
- Her2: Equivocal (score 2+).
- SISH: No residual cells to evaluate.
- Ki67%: 5%.

****Scoring system: "ER, PR"***

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

***** Scoring system: "Her2"***

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

Immunohistochemistry
[[DOCTYPE:IHC]]

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IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghonaim, MD

Immunohistochemistry
[[DOCTYPE:IHC]]

Ministry of Health and Population

Specialized Medical Centers

Nasser Institute Hospital

وزارة الصحة و السكان

أمانة المراكز الطبية المتخصصة

معهد ناصر للبحوث و العلاج

Ultrasound and mammography Breast Report

الاســـــم: جيهان فتحي علي

التاريـــــخ: 2026/06/22

Clinic Back Ground:

- Right breast swelling.

US and mammography FINDINGS:

☒ Prominent fibro glandular tissue of both breast (ACR B).

☒ Prominent fibro-glandular tissue with mottling appearance with:

☒ Right UIQ suspicious mass and non mass area, asymmetry and architectural distortion seen in an area measuring 2.6X2.0 cm seen about 0.3 cm from the overlying skin and about 6 cm from the nipple.

☒ Left UOQ suspicious mass and non mass area, asymmetry and architectural distortion seen in an area measuring 2.6X1.0 cm seen about 0.5 cm from the overlying skin and about 5 cm from the nipple.

☒ Normal left skin nipple and subcutaneous tissue.

☒ Right pathological right axillary LNs (more than 7) and two sub-pectoral LNs.

☒ Few left axillary pathological LNs (about 5)..

Mammogram
[[DOCTYPE:MAMMOGRAM]]

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وزارة الصحة و السكان

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Opinion:

☒ - **Right UIQ suspicious mass and non mass area, asymmetry and architectural distortion with pathological right axillary and sub pectoral LNs (BIRADS 5)**
☒ **Left UOQ suspicious mass and non mass area, asymmetry and architectural distortion with left axillary pathological LNs, (BIRADS 4c).**
☒ **For histopathological correlation.**
BIRADS reporting system:
- Category 0: **Further evaluation is recommended.**
- Category 1: Normal
- Category 2: **Definite benign finding.**
- Category 3: **Probably benign finding.**
- Category 4: **Suspicious lesion, biopsy is a must.**
- Category 5: **Highly suggestive of malignancy, immediate intervention is advised.**

Signature

Dr. / Elham Ahmed, MD

Consultant of Radiology

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient's Name:	**Gihan Fathy Ali Mohammed**
Date:	**2026-08-29**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- Diffuse mosaic attenuation of both lungs ? small airway disease for clinical correlation.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Mild cardiomegaly for echocardiography.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Few gall bladder stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. Prominent left pelvicalyceal system. No stones or right back pressure changes.
- Small splenic hypodense focal lesion measuring 12 mm in diameter.
- Mesenteric panniculitis.
- Normal C.T appearance of the pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- **Small splenic focal lesion.**
- **Otherwise no pulmonary or hepatic metastatic lesions.**

Dr.sherif Wagdy

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	**Gihan Fathy Ali Mohammed**
Date:	**2026-08-29**
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Hospital:	**Goustave Roussey International Egypt**

CT Scan
[[DOCTYPE:CT_SCAN]]

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- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- **Small splenic focal lesion.**
- **Otherwise no pulmonary or hepatic metastatic lesions.**

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